

Name: Brandon Davis | DOB: 3/31/1994 | MRN: 12395948 | PCP: NO PCP | Legal Name: Brandon Davis

Emergency Department - May 23, 2026

at Ssh Emergency

Notes from Care Team

ED Provider Notes by Zachary D Miller at 5/23/2026 9:25 PM

History

Chief Complaint

Patient presents with

- Illness

Pt's sister reported pt has been having some shortness of breath. Family reported he is altered

The history is provided by the patient.

Shortness of Breath

Associated symptoms: **no chest pain**

Patient is a 32-year-old male with history of hypertension who presents to the ED for shortness of breath. Patient reports shortness breath present for 2 years. He reports shortness of breath has been increasing, so he decided to come get evaluated. Patient reports dry, nonproductive cough. He reports shortness of breath increases with exertion. He reports sharp chest pain during these episodes of shortness of breath. He denies any fever, but has had chills. Patient's unaware of any sick contacts. Patient currently does not have a primary care doctor. He reports he's not currently on any blood pressure medication. BP elevated here 170/93 and 175/104. He denies any previous cardiac history. Sister reports he has episodes where he becomes lightheaded and appears to be out of it. No other acute concerns.

Past Medical History

Past Medical History:

Diagnosis

Date

- Hypertensive disorder

Past Surgical History

No past surgical history on file.

Family History

No family history on file.

Social History

Social History

Tobacco Use

- Smoking status: Every Day
- Types: Cigarettes
- Smokeless tobacco: Never

Substance Use Topics

- Alcohol use: Yes
- Drug use: Not Currently

Review of Systems

Respiratory: Positive for **shortness of breath**.

Cardiovascular: Negative for chest pain.

All other systems reviewed and are negative.

Physical Exam

Triage Vitals:

ED Triage Vitals [05/23/26 2012]

Temp	98.7 °F (37.1 °C)
Temp src	Temporal
Heart Rate	100 bpm
BP	(!) 170/93
Resp	18
SpO2	98 %

Physical Exam

Vitals and nursing note reviewed.

Constitutional:

General: He is not in acute distress.

Appearance: He is well-developed. He is not ill-appearing or toxic-appearing.

HENT:

Head: Normocephalic and atraumatic.

Right Ear: External ear normal.

Left Ear: External ear normal.

Eyes:

General:

Right eye: No discharge.

Left eye: No discharge.

Conjunctiva/sclera: Conjunctivae normal.

Cardiovascular:

Rate and Rhythm: Normal rate and regular rhythm.

Pulmonary:

Effort: Pulmonary effort is normal. No respiratory distress.

Breath sounds: Normal breath sounds. No wheezing.

Abdominal:

General: Bowel sounds are normal. There is no distension.

Palpations: Abdomen is soft.

Tenderness: There is no abdominal tenderness. There is no guarding or rebound.

Musculoskeletal:

General: No tenderness.

Cervical back: Neck supple.

Lymphadenopathy:

Cervical: No cervical adenopathy.

Skin:

General: Skin is warm and dry.

Neurological:

Mental Status: He is alert and oriented to person, place, and time.

Psychiatric:

Behavior: Behavior normal.

ED Course

Vitals Recorded in This Encounter

	5/23/2026 2012	5/23/2026 2100
Temp:	98.7 °F (37.1 °C)	—
Temp src:	Temporal	—
Pulse:	100	77
Heart Rate:	100 bpm	—
Resp:	18	—
BP:	170/93 !	175/104 !
MAP (mmHg):	124	127
SpO2:	98 %	94 %
O2 Device:	None (Room air)	—

Procedures

Medical Decision Making

Patient is a 32-year-old male who presents to the ED for shortness of breath. Primary differentials include but are not limited to acute bronchitis versus pneumonia versus COPD.

WBC 13.5. Hemoglobin 18.1. Other labs unremarkable or near baseline. Troponin negative, BNP within normal limits, D-dimer negative.

Chest x-ray shows no acute process.

Patient's blood pressure has been elevated here. He received 1 time dose of labetalol with improvement. Will start patient on losartan 25 mg daily. Recommend patient monitor his blood pressure closely and follow-up with primary care for further management.

Will treat patient for asthma exacerbation. Provided script for ProAir inhaler and Z-Pak. Recommend patient follow-up with primary care for recheck.

Labs Reviewed

COMPREHENSIVE METABOLIC PANEL - Abnormal; Notable for the following components:

Anion Gap	17 (*)
Calcium	10.9 (*)
Albumin	5.6 (*)

All other components within normal limits

CBC (WITH AUTO OR MANUAL DIFFERENTIAL AS INDICATED) - Abnormal; Notable for the following components:

WBC	13.5 (*)
RBC	5.68 (*)
Hemoglobin	18.1 (*)
Hematocrit	52.6 (*)
MPV	7.2 (*)
Neutrophils %	81 (*)
Lymphocytes %	15 (*)
Neutrophils #	11.0 (*)

All other components within normal limits

MG (MAGNESIUM) - Normal

TROPONIN-I - Normal

D-DIMER, QUANTITATIVE - Normal

B-TYPE NATRIURETIC PEPTIDE - Normal

XR Chest 2 View

Final Result

IMPRESSION:

No plain radiographic evidence of acute cardiopulmonary process.

Report dictation location: WJTN2X54

Problem List Items Addressed This Visit

None

Visit Diagnoses

Hypertension, unspecified type - Primary

Relevant Medications

labetalol injection 10 mg (Completed)

losartan (Cozaar) 25 MG tablet

Mild intermittent asthma with exacerbation (HHS/HCC)

Relevant Medications

fluticasone (Flonase) 50 MCG/ACT nasal spray

loratadine (Claritin) 10 MG tablet

albuterol 108 (90 Base) MCG/ACT inhaler

azithromycin (Zithromax) 250 MG tablet

Exam findings and treatment plan discussed with patient. Advised patient to return to emergency department for new or worsening symptoms. Patient verbalized understanding.

Advised patient to follow up with their primary care physician.

The benefits of medications provided today were discussed and reviewed. Take all medication as prescribed. Informed patient if reaction develops to stop medication immediately and to seek treatment.

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Problems Addressed:

Hypertension, unspecified type: complicated acute illness or injury

Mild intermittent asthma with exacerbation (HHS/HCC): complicated acute illness or injury

Amount and/or Complexity of Data Reviewed

Labs: ordered.

Radiology: ordered.

Risk

Prescription drug management.

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05/24/26 0202

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